Medical Coding Reference Guide for Clinical Trial Data Coding: MedDRA, WHODrug, ICH-GCP, and Industry Best Practices

TL;DR

The two dictionaries anchor to two authorities: adverse events and medical history code to **MedDRA** (maintained by the MSSO at meddra.org; current release **v29.0, 1 March 2026**), and concomitant medications code to **WHODrug Global** (maintained by Uppsala Monitoring Centre at who-umc.org; current release **March 2026**). Both require paid, licensed subscriptions, and their principal governance/coding-convention documents (MedDRA Term Selection: Points to Consider; WHODrug User Guide and Best Practices) are freely available reference material.

Regulators enforce version discipline, not just dictionary choice: FDA's Study Data Technical Conformance Guide and Data Standards Catalog require the most current supported MedDRA and WHODrug Global (B3/C3) versions, and a single MedDRA version should be used across a pooled/integrated safety analysis. ICH E6(R3) GCP (adopted 6 January 2025) governs the quality system around coding but does not itself name MedDRA or WHODrug.

Best practice is convention-driven and query-driven: sponsors should maintain a coding conventions manual built on the MTS:PtC, use "always query" rules for ambiguous verbatims, apply autocoding plus manual review with QC (double/independent coding), and plan MedDRA/WHODrug up-versioning at defined study milestones.

Key Findings

MedDRA is governed by ICH through the MSSO. The dictionary is a subscription product; the browsers (Web-Based, Desktop, Mobile) and support documents (Introductory Guide, Term Selection: Points to Consider, Data Retrieval and Presentation: Points to Consider, SMQ Introductory Guide, What's New) are distributed to subscribers, with many documents also posted publicly on meddra.org.

MedDRA releases twice yearly — a major version each March (x.0) and a minor version each September (x.1). Recent releases: v27.0 (1 Mar 2024), v27.1 (Sep 2024), v28.0 (1 Mar 2025), v28.1 (Sep 2025), v29.0 (1 Mar 2026).

WHODrug Global is maintained by UMC, released biannually (1 March and 1 September), distributed in the **B3 and C3 formats**, with the FDA-mandated B3/C3 formats required for study submissions since March 2019. WHODrug Koda is UMC's AI/ML automated coding engine.

ICH E6(R3) GCP is the current global GCP standard (ICH-adopted 6 Jan 2025; EU effective 23 Jul 2025; FDA final guidance 9 Sep 2025); its Annex 2 reached Step 4 in June 2026 (EU effective 15 Jan 2027).

Coding conventions should be organization-specific but consistent with the ICH-endorsed Points to Consider documents, addressing misspellings, abbreviations/acronyms, combination/"due to" terms, and "always query" terms (e.g., "chest pain").

Details

1. MedDRA (Medical Dictionary for Regulatory Activities)

Governance and access. MedDRA is a subscription product of ICH, maintained by the Maintenance and Support Services Organization (MSSO). It is free to regulatory authorities, non-profits, direct patient care providers and educational institutions, and charged on an annual subscription to commercial users. Japanese-headquartered organizations subscribe through the Japanese Maintenance Organization (JMO). A licence is required not only to code but to *receive* coded terms (data transfers, listings, coding reports, terms in submissions such as DSURs, PSURs/PBRERs, CSRs).

MSSO organization page: https://www.meddra.org/about-meddra/organisation/msso
Subscription process: https://www.meddra.org/subscription/process
Subscription types: https://www.meddra.org/subscription/subscription-type
Special licences (regulators / low-revenue / EVWEB fee-waiver): https://www.meddra.org/subscription/special-licences
Subscription portal: https://subscribe.meddra.org/

Browsers (subscriber access). Three browsers are provided:

Web-Based Browser: https://tools.meddra.org/wbb/ (and legacy http://mssotools.com/mssoweb/wbb/wbb_index.html)
Desktop Browser: https://www.meddra.org/meddra-desktop-browsers
Mobile Browser: https://mmb.meddra.org
Browser overview: https://www.meddra.org/browsers

The browsers include the full hierarchy in all supported languages and allow searching at any level or across combinations of levels.

Hierarchy structure (SOC → HLGT → HLT → PT → LLT). MedDRA is a five-level, multi-axial hierarchy:

SOC (System Organ Class) — broadest level; there are 27 SOCs, grouped by etiology, manifestation site, or purpose.
HLGT (High Level Group Term)
HLT (High Level Term)
PT (Preferred Term) — the principal coding level for AEs; represents a single medical concept (symptom, sign, disease, diagnosis, indication, investigation, procedure, or social/medical-history characteristic).
LLT (Lowest Level Term) — most specific; the level to which a verbatim is typically matched. Each PT has at least one LLT (including a term identical to the PT). Non-current LLTs (vague, ambiguous, outdated, truncated, misspelled, or derived from other terminologies) are retained for historical retrieval but not recommended for continued use.

Each PT links to at least one SOC but is assigned exactly one **primary SOC** for standardized output; multi-axiality means a PT can appear under multiple SOCs. Each MedDRA term carries an 8-digit numeric code.

Introductory Guide (current v29.0, March 2026):
https://alt.meddra.org/files_acrobat/intguide_29_0_English.pdf (historical example:
https://admin.meddra.org/sites/default/files/guidance/file/intguide_21_0_english.pdf)
"Understanding Coding with MedDRA" (MSSO training):
https://admin.meddra.org/sites/default/files/page/documents_insert/

Version release history and cadence. Major versions release each March (x.0), minor each September (x.1). Recent:

v27.0 — 1 March 2024 (1,582 change requests; 1,206 approved/implemented)
v27.1 — September 2024
v28.0 — 1 March 2025 (English 1 Mar; translations 15 Mar 2025)
v28.1 — September 2025
v29.0 — 1 March 2026. Per SafetyDrugs (Sonia Monni, 7 May 2026), v29.0 "considered 1,380 change requests, of which 1,042 were approved and implemented," adding **611 new LLTs and 198 new PTs**. It is a "complex change" version; the only approved complex change is at HLT level ("the introduction of the new term Poliovirus infections and the concurrent retirement of the HLT Poliomyelitis virus infections"), with net SOC impact concentrated in "Investigations, with 118 net changes, Injury, poisoning and procedural complications, with 114, and Product issues, with 99." v29.0 also added the new PT "Suspected off label use" and updates to the MTS:PtC on off-label scenarios. MedDRA now supports **28 languages** with the addition of Danish, with Bulgarian, Maltese, Romanian and Uzbek under development.
Version news examples: https://www.meddra.org/news-and-events/news/english-meddra-version-280-now-available
"What's New" release notes (e.g., v27.0):
https://admin.meddra.org/sites/default/files/guidance/file/whatsnew_27_0_English.pdf
What's New webinars: https://files.meddra.org/www/Training%20Materials/

Points to Consider (PtC), Introductory Guide, SMQs.

MedDRA Term Selection: Points to Consider (MTS:PtC) — ICH-endorsed term-selection guide, updated annually in step with the March release (since v23.0). Developed by an ICH Management Committee working group (ICH regulators + industry, WHO, MSSO, JMO). HTML: https://alt.meddra.org/htmlPages/ts_ptc_r422.html
Data Retrieval and Presentation: Points to Consider (DRP:PtC) — companion guide on data retrieval/presentation and SMQ use. PDF: https://alt.meddra.org/files_acrobat/001007_datretptc_r3_24_mar2024.pdf ; HTML: https://files.meddra.org/www/Website%20Files/PtCs/drp_ptc_r324.html
Introductory Guide (v29.0, March 2026): https://alt.meddra.org/files_acrobat/intguide_29_0_English.pdf
Standardised MedDRA Queries (SMQs) — pre-defined groupings of PTs for a medical condition, with narrow (specificity) and broad (sensitivity) scopes, some hierarchical (sub-SMQs), some algorithmic. Landing page: https://www.meddra.org/how-to-use/tools/smqs ; SMQ Introductory Guide (v27.0): https://admin.meddra.org/sites/default/files/guidance/file/SMQ_intguide_27_0_English.pdf . The CIOMS "Development and Rational Use of SMQs" report (2nd ed., 2016, the "Red Book") is a companion reference. SMQs originated with a CIOMS Working Group (early 2000s) and typically comprise ~50–200 PTs.

SMQ content is distributed as an Excel spreadsheet with each release (PT name, PT code, SMQ category A/B/C, scope narrow/broad).

Change request and version-comparison tools.

WebCR — web application for submitting change requests (term, SMQ, translation), limit 100/month (translations unlimited); records all change requests from v5.1 to present. Change request info: https://www.meddra.org/how-to-use/change-requests
MVAT (MedDRA Version Analysis Tool) — free to subscribers; produces the Version Report comparing consecutive versions (net changes per SOC, term name changes, LLT currency changes). Weekly supplemental update files preview approved changes for the next release.
What's New document accompanies each release (posted under Support Documentation).
MSSO also provides ICD-10↔MedDRA and IMDRF↔MedDRA mappings and the Important Medical Event (IME) list.

Coding conventions / MSSO best-practice guidance (per MTS:PtC).

Select the current LLT that most accurately reflects the verbatim (e.g., "Abscess on face" → "Facial abscess," not simply "Abscess"); if no exact match, use medical judgment to match an existing term that adequately represents the concept.
Do not alter MedDRA or make ad hoc structural changes (including changing the primary SOC allocation); submit a change request instead.
Select only current LLTs (non-current terms for legacy/historical conversion only).
Obtain clarification for ambiguous/confusing/unintelligible data; avoid company-specific "work-arounds."
Check the hierarchy above a selected LLT (PT/HLT/HLGT/SOC) to confirm placement is appropriate.
The MTS:PtC gives specific guidance for misspellings, abbreviations and acronyms, combination terms and "due to" concepts, and "always query" terms (e.g., "chest pain"); do not infer malignancy or circumstances of death not stated by the reporter. A companion document with detailed data-quality examples exists.

2. WHODrug (WHO Drug Dictionary / WHODrug Global)

Governance and access. WHODrug Global is UMC's drug dictionary (UMC = WHO Collaborating Centre for International Drug Monitoring, Uppsala, Sweden, since 1978). Subscriptions are available to commercial and non-profit organizations, system developers/vendors, regulatory authorities, and National Pharmacovigilance Centres; NPCs in the WHO Programme receive it free for non-commercial use. Use of WHODrug data requires a valid subscription; the documentation is CC BY 4.0 licensed. Who-umc + 2
↗

UMC home: https://who-umc.org/
WHODrug Global product page: https://who-umc.org/whodrug/whodrug-global/what-is-whodrug-global/
Subscription: https://who-umc.org/whodrug/whodrug-subscription/ ; types: https://who-umc.org/whodrug/whodrug-subscription/subscription-types/
Applications & services: https://who-umc.org/whodrug/whodrug-global/applications-and-services/
Download Area (biannual releases, 1 March / 1 September): https://who-umc.org/whodrug/whodrug-global/applications/download-area/

WHODrug User Guide (March 2026 edition). Full title "WHODrug User Guide — Applicable for the B3 and C3 formats," March 2026. URL: https://who-umc.org/media/k1wmz52z/whodrug-user-guide-march-2026.pdf. Main sections: Content; Disclaimer; Changes from previous version; Getting started (product portfolio: terminology, SDG, Koda, Cross Reference ATC 5, CRT Japan, Link Korea); WHODrug Global (terminology, formats); Core WHODrug concepts (Drug name, Drug code, C3-specific concepts); WHODrug Global Chinese; ISO IDMP data and mappings; Accessing WHODrug data (Insight, file packages); ATC classification; Changes to the WHODrug data (CAT, Change Request). Key concept definitions:

Drug code — alphanumeric aggregation of Drug Record Number (DrugRecNo) + Sequence number 1 (Seq1) + Sequence number 2 (Seq2); "uniquely identifies a drug name and its substance(s) or an Umbrella record"; the unique identifier in the B3 format.
Drug Record Number — first six characters; "uniquely identifies the substance, or combination of substances, regardless of variation or plant part." Seq1 (2 chars) identifies substance variations (01 = base); Seq2 (3 chars) identifies the record name (001 = substance; >001 = product names/synonyms). who-umc ↗
ATC classification — hierarchical classification of active substances by organ/system and therapeutic/pharmacological/chemical properties; integrated in WHODrug; all records assigned at least one ATC code (preferably 4th-level); codes flagged "official" (WHO CC Oslo index) or "umc-assigned"; ATC 5th-level not included in WHODrug (the Drug code fills that identifier role). UMC-created codes include V90 (unspecified herbal/traditional), V91 (homeopathic), V98 (investigational drug). who-umc + 4 ↗
Substance name / INN — NOTE: the March 2026 edition renamed "Preferred name" to "substance name." Substance name is generally an INN; if none exists, another globally recognized nonproprietary name (USAN, JAN) or chemical/botanical name is used. Additional generic records for a substance are "substance synonym names" (e.g., Paracetamol = INN; Acetaminophen = synonym). For all WHODrug records the substance/preferred name is always generic. who-umc ↗
who-umc ↗
Umbrella records — records "representing a drug category or a placeholder record rather than a specific drug name" (e.g., Corticosteroids, Placebo), used for coding imprecise verbatims when specific coding is not possible; Drug code starts with 9; always lack ingredients; two types (ATC Umbrella, UMC-created Umbrella).
Multi-ingredient products — ingredients listed alphabetically, semicolon-separated; in B3, non-unique names are appended with [INGREDIENT(S)]; "most multi-ingredient records in WHODrug have UMC-assigned ATC codes" (combinations rarely get official ATC). who-umc ↗
Archived record (renamed from "Old form" in March 2026) — reflects "changes in medicinal product compositions, product withdrawal, or removal from market"; both old and new records remain available for coding; in C3, indicated in the "Archived Record" additional feature file; the least specific RID (trade name + substance) is never archived.

Coding guidance / Best Practices.

WHODrug Best Practices (B3/C3), v6.0: https://edit.who-umc.org/media/hwlbrgep/whodrug-best-practices-v-6-0-applicable-for-the-b3-and-c3-formats2.pdf (v5.0: https://who-umc.org/media/3128/whodrug-best-

practices-vers-50_new.pdf)
How to use WHODrug C3 format for drug coding — coding decision flow (code to the least specific record when only a brand name is reported; to the active ingredient record when only ingredient is reported; seek reporter clarification when insufficient). Gobierno de México ↗
Documentation landing page: https://who-umc.org/whodrug/documentation/ — links to User guides (https://who-umc.org/whodrug/documentation/guides/), Coding guidance (https://who-umc.org/whodrug/documentation/coding-guidance/), WHODrug developments (https://who-umc.org/whodrug/documentation/whodrug-developments/), Supplementary documents incl. CDISC SDTM compliance (https://who-umc.org/whodrug/documentation/supplementary-documents/), File descriptions (https://who-umc.org/whodrug/documentation/file-descriptions/), Version verification (https://who-umc.org/whodrug/documentation/version-verification/), and Industry eReporting (https://who-umc.org/whodrug/documentation/industry-ereporting/).

ATC classification resources (external authority). The ATC/DDD system is maintained by the **WHO Collaborating Centre for Drug Statistics Methodology (Oslo, Norwegian Institute of Public Health).** Five levels: 1st = anatomical/pharmacological main group (14 groups); 2nd = therapeutic/pharmacological subgroup; 3rd & 4th = chemical/pharmacological/therapeutic subgroups; 5th = chemical substance (e.g., metformin = A10BA02). Updated annually. National Library of Medicine + 2 ↗

Searchable ATC/DDD Index: https://atcddd.fhi.no/atc_ddd_index/
Guidelines for ATC classification and DDD assignment: https://atcddd.fhi.no/atc_ddd_index_and_guidelines/guidelines/
WHO ATC-DDD Toolkit: https://www.who.int/tools/atc-ddd-toolkit ; ATC classification page: https://www.who.int/tools/atc-ddd-toolkit/atc-classification
UMC's "Classifications in WHODrug: ATCs and SDGs" (3 Oct 2023): https://who-umc.org/media/1grndglz/classifications-in-whodrug-atcs-and-sdgs-20231003.pdf — NOTE: this specific PDF requires UMC login/subscription; its full content on ATC vs SDG could not be independently verified. In summary form, ATC codes reside in the ATC/DDD index while SDGs reside within WHODrug Global. Who-umc ↗

WHODrug SDGs (Standardised Drug Groupings). Per the User Guide (§4.2.2): "WHODrug Standardised Drug Groupings (SDGs) are standardised and unbiased groupings of drugs, that can identify drugs with the same properties. The SDGs can be used for example in various types of safety analysis, or in the specification of exclusion criteria in clinical trials. All SDGs allow for modifications by the user, and when altered they are referred to as Customised Drug Groupings (CDGs)." Included with a WHODrug Global subscription.

Release cadence and version naming. WHODrug Global is released biannually (1 March, 1 September). Versions are named by release date (e.g., "March 2026"). The March 2026 "What's New" states the release includes "more than 688,000 unique product names and over 5 million medicinal products." (Country coverage is cited inconsistently across UMC materials — the March 2026 "What's New" cites 175 countries, while UMC's WHODrug Global product page states data "from nearly 150 countries"; use the UMC product-

page figure of ~150 unless quoting the What's New document directly.) Who-umc ↗

Format versions (B3, C3). WHODrug is distributed in two formats; both are provided to all users:

B3 — drug names, active substances, ATC classifications; unique identifier is the Drug code. In B3, generic records inherit all ATC codes of linked products; non-unique trade names are appended with [INGREDIENT(S)]. who-umc + 2 ↗

C3 — all B3 information plus country of sale, marketing authorisation holder (MAH), pharmaceutical form (EphMRA NFC classification), and strength; "higher precision of ATC code assignments since each drug name is assigned specific ATC codes"; unique identifier historically the WHODrug MPID (renamed **Record ID (RID)** in March 2026).

The B3/C3 formats replaced the older B2/C formats. UMC first released B3/C3 in March 2017 and ran B2/C in parallel until 1 March 2019; the FDA required B3 for study submissions starting after 15 March 2019. Learning-scdm ↗

B3/C3 implementation guide (2018): https://edit.who-umc.org/media/164798/whodrug-b3-and-c3-formats-implementation-guide-april-2018.pdf
WHODrug Global Structural Changes 2026: https://who-umc.org/media/w11lai1c/whodrug-global-structural-changes-2026-v3-0.pdf
What's New in WHODrug 2026: https://who-umc.org/media/5nva5npw/what-s-new-in-whodrug-2026.pdf (March 2026 changes: ISO IDMP data/links added; "Preferred records"→"Substance records", generic records with drug code ending >001 →"Substance synonym records", "Old form"→"Archived record", "MPID"→"Record ID (RID)"; .txt files discontinued in favor of .csv "because .txt files cannot manage large data sets and non-Latin characters well"; WHODrug User Area removed with all documents moved to the documentation section; WHODrug Insight "MPID Search" renamed "Extended Drug Search" with wildcard (*) search; new "UMC Product ID" for the most specific C3 record; ATC annual revision adding 3 new 4th-level and 96 new 5th-level codes).

WHODrug Koda. UMC's automated drug-coding engine (developed 2019), combining text-processing algorithms, built-in coding rules, and machine learning; provides automated drug-name coding (including non-unique names) and automated ATC selection per current regulatory expectations. Available as a REST-based HTTP API and as a web application; included in the standard WHODrug Global subscription (free for vendors in the WHODrug Vendor Programme). Peer-reviewed evaluation — Meldau, Bista, Rofors & Gattepaille, *Drug Safety* 2022 May;45(5):549–561, "Automated Drug Coding Using Artificial Intelligence: An Evaluation of WHODrug Koda on Adverse Event Reports": "Compared with a simple direct-match baseline, Koda can increase the automation level from 61% to 89%, while providing high coding quality with an accuracy of 97%." Azurewebsites + 2 ↗

Koda page: https://who-umc.org/whodrug/whodrug-koda/
Koda for vendors: https://who-umc.org/whodrug/whodrug-koda/implementing-whodrug-koda-for-vendors/

Change Analysis Tool (CAT) — impact analysis / up-versioning tool comparing versions (including B2→B3 comparison); free to subscribers with a personal UMC login. https://who-umc.org/whodrug/whodrug-global/applications/whodrug-cat/

3. ICH-GCP (E6(R2) and E6(R3))

Hosting and status. ICH hosts GCP guidelines at database.ich.org.

E6(R3) — final Step 4 guideline (Principles + Annex 1), adopted 6 January 2025: https://database.ich.org/sites/default/files/ICH_E6(R3)_Step4_FinalGuideline_2025_0106.pdf . EU (EMA/CHMP) effective 23 July 2025; FDA final guidance issued 8 September and announced in the Federal Register 9 September 2025; Health Canada implementation 1 April 2026. Annex 2 reached Step 4 (ICH 3 June 2026; CHMP 25 June 2026), EU effective 15 January 2027.
EMA E6 scientific guideline page: https://www.ema.europa.eu/en/ich-e6-good-clinical-practice-scientific-guideline
FDA E6(R3) guidance / Federal Register: https://www.federalregister.gov/documents/2025/09/09/2025-17311/e6r3-good-clinical-practice-international-council-for-harmonisation-guidance-for-industry
E6(R2) — the prior Integrated Addendum (adopted 2016), still cited during the transition; the ICH E6 page and database host both.

Relevance to safety data coding. E6(R3) is a principles-and-quality guideline: it addresses data governance, computerized systems, data integrity/traceability, risk-proportionate quality management, and the safety-reporting/assessment responsibilities of sponsors and investigators. Query management (resolving ambiguous/inconsistent data) is a regulatory expectation under E6(R3) Principles and Annex 1.

References to MedDRA / WHODrug. ICH E6(R3) does **not** name MedDRA or WHODrug or mandate a specific coding dictionary. Standardized medical-terminology requirements come from other ICH guidelines and regional rules: ICH M1 governs MedDRA itself; ICH E2A/E2B/E2D (adverse-reaction reporting and E2B(R3) electronic transmission) and EU GVP Module VI operationalize MedDRA-coded ICSR reporting; FDA's Data Standards Catalog and Study Data Technical Conformance Guide mandate MedDRA and WHODrug Global for study-data submissions.

4. Sponsor SOP conventions, industry best practice, and regulatory version expectations

CDISC SDTM mapping. Coded terms flow into SDTM domains: **AE** (adverse events, MedDRA — e.g., AEDECOD = PT, AELLT, AEBODSYS = SOC), **MH** (medical history, MedDRA), **CM** (concomitant medications, WHODrug). UMC's guide "How to use WHODrug for compliance with the CM domain in the CDISC SDTM standard" (based on SDTM v2.1 / IG v3.4) identifies the key CM variables CMTRT, CMMODIFY, CMDECOD, CMCLAS, CMCLASCD, and warns that where multiple ATC codes exist the single submitted ATC "must be manually selected based on information available on the CRF from the investigator... It is not recommended to randomly select one ATC code."

UMC CM/SDTM guide: https://who-umc.org/media/1ldnsbuk/how-to-use-whodrug-for-compliance-with-cm-domain-in-the-cdisc-sdtm-standard-v2-0.pdf
CDISC standards: https://www.cdisc.org

Coding conventions manuals and query management. Industry best practice (CROs, CDM organizations, and MSSO/UMC guidance) is to maintain an organization-specific **coding conventions manual** consistent with the Points to Consider documents, covering misspellings, abbreviations, combination terms, and "always query" scenarios. Source-data quality is foundational: investigators should be trained to report precise verbatims and avoid vague descriptions (e.g., "felt bad"). Autocoding (direct-match plus ML engines — WHODrug Koda, Medidata Rave Coder, Oracle Thesaurus Management System, CluePoints) is combined with manual review of unmatched/ambiguous verbatims, and medical coders raise queries to sites for clarification. QC methods include independent/double coding and risk-based QC review, with a documented audit trail of every coding decision.

FDA / EMA version-consistency expectations.

FDA supports and requires the most current MedDRA version for studies (transition rule: support from March 2018, requirement for studies starting after 15 March 2019), and B3/C3 WHODrug Global for submissions after 15 March 2019; details in the **Study Data Technical Conformance Guide** and Data Standards Catalog. Current sdTCG editions include March 2025 (v6.0), December 2025, and June 2026 (v6.2.1). sdTCG: https://www.fda.gov/media/88173/download
For an Integrated Summary of Safety (ISS), a **single MedDRA version** should be used across pooled studies "to ensure that the same adverse events reported in different studies are classified consistently"; the pooled datasets support Module 5 of the eCTD. Both MSSO and UMC require an active licence for any party that uses *or receives* coded terms.
EMA GVP Module VI requires MedDRA-coded ICSRs submitted electronically to EudraVigilance via ICH E2B(R3); the EMA Important Medical Event (IME) list is updated in line with each MedDRA version.
Practical guidance on up-versioning timing: https://www.precisionformedicine.com/blog/when-to-up-version-your-meddra-and-whodrug-dictionaries/

Recommendations

Set up licensing first. Confirm active MedDRA (MSSO/JMO) and WHODrug Global (UMC) subscriptions for every party that codes *or receives* coded terms (sponsor, CRO, safety vendor, biostatistics). Absence of a licence for any downstream recipient is a compliance gap explicitly called out by both MSSO and UMC.
Codify a coding conventions manual built on the current **MedDRA Term Selection: Points to Consider** and **WHODrug Best Practices**, with explicit rules for misspellings, abbreviations, combination/"due to" terms, and an "always query" list (e.g., "chest pain"). Reference the DRP:PtC and SMQ Introductory Guide for retrieval/analysis conventions, and align SOPs to the v29.0 MTS:PtC update on suspected off-label use.
Freeze dictionary versions per study and plan up-versioning at milestones (e.g., database lock, before an ISS/ISE). Use one MedDRA version across a pooled safety analysis. Use **MVAT** (MedDRA) and **WHODrug CAT** (UMC) for impact analysis before any mid-study up-version, and document the rationale.
Combine autocoding with manual QC. Deploy an autocoder (e.g., WHODrug Koda for CM; a MedDRA autocoder for AE/MH) for direct hits, route ambiguous terms to trained coders, and apply independent/double coding QC on a risk-based sample. Retain an audit trail of every coding decision (an E6(R3) and EMA-inspection expectation).

Align to submission standards. Map to CDISC SDTM (AE/MH → MedDRA; CM → WHODrug B3/C3), confirm the FDA Data Standards Catalog-supported versions before submission, and validate with conformance tooling (e.g., Pinnacle 21) against the current rules.

Benchmarks that would change these steps: a new March/September MedDRA or WHODrug release during the study (triggers an up-version impact assessment); a new FDA sdTCG/Data Standards Catalog edition changing supported versions; adoption of ICH E6(R3) Annex 2 (EU effective 15 Jan 2027) affecting decentralized/RWD data handling; or a regulatory request (e.g., for a specific SMQ analysis) requiring recoding to a specified version.

Caveats

Paid vs public: the MedDRA and WHODrug *terminologies/data* and their browsers/tools (Web-Based/Desktop/Mobile browsers, MVAT, WebCR, WHODrug Insight, CAT, Koda) require paid, licensed subscriptions. Many *documents* (Introductory Guide, MTS:PtC, DRP:PtC, SMQ Introductory Guide, What's New, WHODrug User Guide, Best Practices) are freely downloadable, but some UMC files (e.g., the 3 Oct 2023 "Classifications in WHODrug: ATCs and SDGs" PDF) sit behind a UMC login. Do not assume a document is public without checking.

Do not fabricate codes: this guide intentionally describes dictionary structure and reference materials only; specific MedDRA PT/LLT codes or WHODrug Drug codes must be taken from the licensed dictionaries at the version in use.

Terminology drift: WHODrug's March 2026 edition renamed several core concepts ("Preferred name"→"substance name", "Old form"→"Archived record", "MPID"→"Record ID/RID"). Pre-2026 documents use the prior terms; reconcile terminology to the version actually loaded.

Conflicting figures: the WHODrug country-coverage figure differs between UMC's own materials (175 countries in the March 2026 "What's New" vs "nearly 150 countries" on the product page). The v29.0 count of ">81,000 LLTs" traces only to a secondary source (IntuitionLabs, 20 Nov 2025), not an MSSO primary document; the exact v29.0 net additions (611 LLTs, 198 PTs; 1,042 approved change requests) come from SafetyDrugs' reading of the v29.0 What's New. Confirm precise counts in the official MSSO v29.0 What's New / Introductory Guide before quoting in a submission.

Some secondary sources are commercial/blog content (CRO and vendor blogs, LinkedIn). Where used, they corroborate primary MSSO/UMC/ICH/FDA/EMA sources rather than serve as the authority; always defer to the primary documents linked above.